

Intuitive Surgical

NASDAQ : ISRG · Mature-Company DCF

\$378.81
\$134.44B mkt cap · 355M sh · \$8.63B cash, zero debt · 11,710 da Vinci installed base

THE CENTRAL QUESTION

Intuitive's soft-tissue near-monopoly — 11,710 da Vinci systems, ~78% recurring revenue — now trades at ~38× forward P/FCF after the market priced in part of the bear's compression. Does that multiple over-discount erosion (Hugo, Ottava, GLP-1s) that Q2 actuals don't yet show, or is the compression only beginning?

Intuitive trades at \$345.74 (\$122.7B cap) — down ~21% since May, including -14.1% on Jul 17: Q2 beat every line (revenue +19%, non-GAAP EPS +28%, procedures +16%, margin guide raised) yet guidance was steered to the midpoint, implying H2 deceleration. Still: ~78% recurring revenue, 11,710 systems, \$8.63B cash, zero debt. But the bear is live: Hugo cleared US urology (Dec 2025), GS/GYN 510(k)s filed (Jun 2026); Ottava passed FORTE, FDA nod reported in July; GLP-1s shave bariatric volumes; a May 2026 Class I recall dented the record. Mature-Company DCF with exit-multiple terminal framing below; five scenarios, weighted; show your work.

PROBABILITY-WEIGHTED EXPECTED VALUE

\$504.14 expected fair value today
+33.1% vs spot **\$378.81**

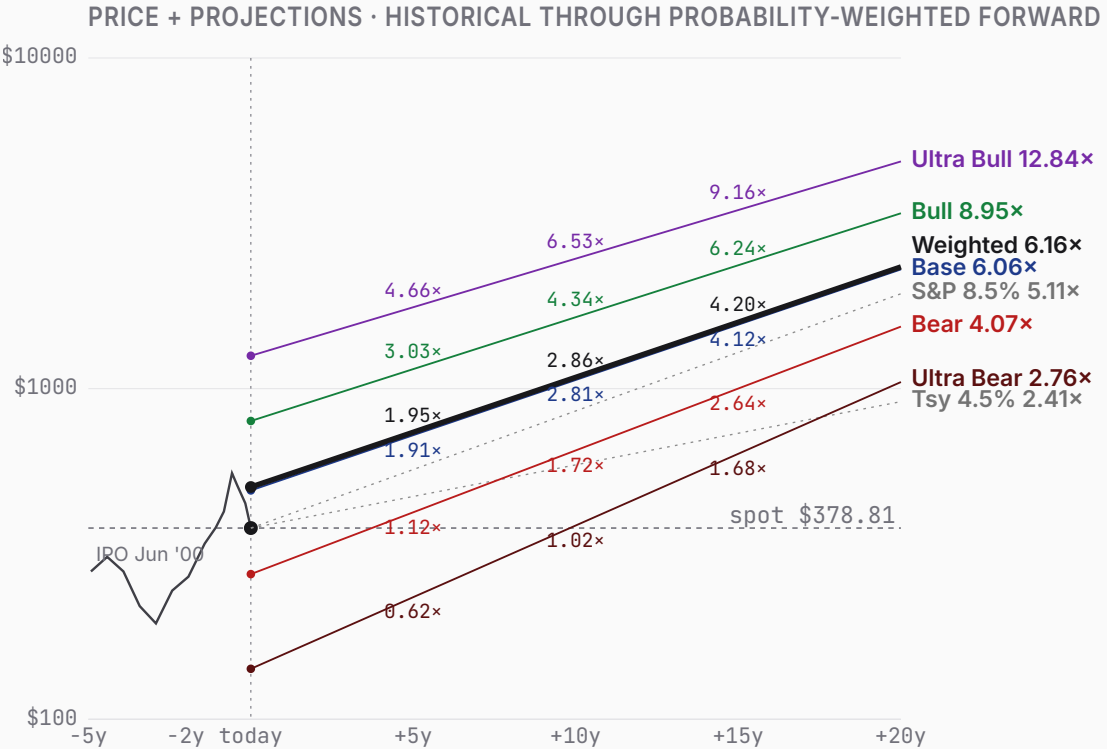
FORWARD COMPOUNDED VALUE

+5y	+10y	+15y	+20y
\$738.53	\$1,082.90	\$1,589.40	\$2,335.16
1.95× spot	2.86× spot	4.20× spot	6.16× spot

WEIGHTING RATIONALE

- Ultra Bear 6%** GLP-1s + Hugo + Ottava + recall cycle compound; multiple compresses to ~28× FCF.
- Bear 25%** Hugo lands at 8-12% of new placements; multiple resets to ~33×; revenue decelerates to 5% by FY30.
- Base 48%** Mid-teens growth holds; dV5 reaccelerates utilization; recurring revenue compounds; multiple ~45×.
- Bull 16%** dV5 wins general-surgery TAM; lon + cardiac add growth; multiple sustains at ~52×.
- Ultra Bull 5%** Outside-OR + China + AI inflect simultaneously; multiple re-rates to ~62×.

Bull (16%) + Ultra Bull (5%) together contribute \$190.61 — 38% of the \$504.14 expected value despite 21% combined probability. Today's spot (\$378.81) sits 25% below the weighted expected; forward-weighted value crosses spot between +5y and +10y.



ULTRA BEAR	\$142.15	BEAR	\$274.86	BASE	\$492.27	BULL	\$798.36	ULTRA BULL	\$1257.39
	-62.5% vs spot		-27.4% vs spot		+30.0% vs spot		+110.8% vs spot		+231.9% vs spot
CAGR +5.2% WACC 10.5% Base 14.5K Prob 6%		CAGR +9.3% WACC 9.0% Base 16.5K Prob 25%		CAGR +12.0% WACC 8.0% Base 18.5K Prob 48%		CAGR +15.0% WACC 7.5% Base 21.0K Prob 16%		CAGR +18.8% WACC 7.0% Base 25.0K Prob 5%	
GLP-1s + Hugo + recall.		Hugo lands; multiple resets to 33×.		Mid-teens growth; razor compounds.		dV5 + general surgery wins; 52× holds.		Outside-OR + China + AI; 62× re-rate.	



Intuitive Surgical scenario narratives

PROBABILITY WEIGHTS

Ultra Bear 6% Bear 25% Base 48% Bull 16% Ultra Bull 5% · Weighted expected \$504.14 (+33.1% vs spot)

ULTRA BEAR	\$142.15	BEAR	\$274.86	BASE	\$492.27	BULL	\$798.36	ULTRA BULL	\$1257.39
	-62.5% vs spot		-27.4% vs spot		+30.0% vs spot		+110.8% vs spot		+231.9% vs spot

Probability 6%

GLP-1s + Hugo + recall.

WHY 6%

Joint conjunction of four largely independent risks: (a) GLP-1 bariatric drag at the high end — this scenario needs ~30-40% volume compression, while Q2 2026 put the observed US bariatric decline at high-single-digits; (b) Hugo + Ottawa combined share gain >15% over five years, requiring both to scale and hospitals to multi-source; (c) a quality/recall cycle that compresses the multiple structurally; (d) core-specialty (urology, GYN) procedure mix maturing earlier than expected. Each individually 15-25% conditional; joint stays at 6% — the conjunction requires multiple moats to break simultaneously within five years, faster than incumbents typically lose share in regulated medtech (historical base rate: 8-12 years for a soft-tissue leader to lose 20+ points of share).

WHAT HAPPENS

The ultra-bear is a coordinated shock, not slow erosion. GLP-1 penetration compresses US bariatric volumes 30-40% over five years; Medtronic Hugo wins meaningful urology share (15-20% of new placements by FY28) and gets general-surgery clearance FY27; J&J Ottawa launches commercially FY27 and takes 5-8% of share; and a recurring quality/recall cycle (the May 2026 SureForm 30 Class I recall was the warning shot) compresses operator confidence.

Procedure growth collapses from 17% to ~6% by FY28, then 2% terminal. Installed-base growth halves from 12% to 5-6%. System ASPs compress as competitive bids force Xi-account discounting. Operating margin compresses from 29% to 22% on defensive spend. The multiple resets toward 28× FCF as the "monopoly compounder" narrative breaks. Exit-multiple SOTP: \$42B op EV + \$8.63B cash = \$51B equity ÷ 358M shares = ~\$142/share — a -59% tail.

Probability 25%

Hugo lands; multiple resets to 33×.

WHY 25%

The competitive landscape genuinely shifted: Hugo's Dec-2025 urology clearance was followed six months later by GS + GYN 510(k) filings (Jun 2026), and Ottawa's FORTE pivotal met its endpoints. History suggests soft-tissue medtech incumbents lose 5-15 points of share over five years when the first credible competitor lands (analog: Stryker Mako vs Zimmer ROSA in ortho). The weight rises from 20% to 25% on management's own words, not price action: FY26 procedure guidance was held but steered to the midpoint — H2 deceleration from H1's ~15% — with US da Vinci +12% vs +16% worldwide, US bariatric down high-single-digits on GLP-1s, and the ACA enhanced-subsidy expiration a named "modest adverse impact." Q2 actuals (procedures +16%, dV5 246 placements) say we're not in this scenario yet; bear is the FY27-28 thesis, and the multiple reset toward 33× is the dominant driver.

WHAT HAPPENS

The pessimistic case takes the new competitive landscape seriously. Medtronic Hugo received FDA urology clearance in December 2025 — the first credible US soft-tissue robotics competitor in 25 years — and filed general-surgery and GYN 510(k)s just six months later (Jun 2026), the fast end of the clearance clock. Hugo takes 8-12% of new system placements by FY28, primarily at price-sensitive community hospitals. J&J's Ottawa — FORTE pivotal met, FDA nod reported Jul 2026 — erodes the "no real competition" narrative; CMR Versius takes multi-specialty community share.

Revenue growth decelerates from 19% (Q2 2026) toward 9% by FY28 and 5% by FY30 as urology + gynecology mature and general surgery competes with Hugo on price. Operating margin holds at 27-29% — Hugo competitive-bid gross-margin pressure offset by mix toward higher-margin instruments/services on the installed base. The multiple resets from ~55× toward 33× FCF.

Probability 48%

Mid-teens growth; razor compounds.

WHY 48%

Modal outcome. Q2 2026 beat every line — revenue \$2.89B +19%, non-GAAP EPS +28%, procedures +16%, gross-margin guide raised to 68-69% — and dV5 placements accelerated again (246 after Q1's 232). The trajectory is in motion, not a future call. The weight trims from 52% to 48% because management held FY26 guidance but steered to the midpoint (H2 deceleration from H1's ~15%) and Hugo's clearance clock is running faster than "slow" — GS/GYN 510(k)s landed six months after urology. Still the mature-band modal weight (ZM 50, LULU 45): the moat (surgeon training, switching cost, FDA data) provides more inertia than seat-based SaaS contracts, and nothing in Q2 actuals validates the bear yet.

WHAT HAPPENS

The middle path: ISRG executes on a near-monopoly that erodes slowly, not catastrophically. FY26 guidance (13.5-15.5% procedure growth) lands at the midpoint management now points to. da Vinci 5 keeps ramping — 246 placements in Q2 2026, toward ~5,000 systems by FY30 — driving a mid-single-digit utilization tailwind on the installed base. Ion scales (+36% Q2 procedure growth annualizes toward 25-30%). Cardiac clearance opens a ~\$2B incremental specialty.

Hugo lands, but at the pace medtech rollouts actually run: 5-7% of new placements by FY28, mostly at community hospitals Intuitive was always going to fight for. The installed-base switching cost — surgeon retraining, instrument inventory, service contracts — keeps the 11,000+ existing systems sticky. Recurring mix climbs from 78% to 81% as services and instruments compound on installed-base growth.

Probability 16%

dV5 + general surgery wins; 52× holds.

WHY 16%

da Vinci 5 captures general surgery (vs Hugo's urology-first rollout) AND ISRG holds 85%+ of incremental US placements through FY28 AND Ion scales to ~\$1.5B revenue AND the multiple sustains at ~52× FCF. The 16% weight is reasonable because (a) dV5 placement velocity keeps rising (246 in Q2 2026 after 232 in Q1, vs 147 a year ago) and the GS mix shift shows penetration, (b) general surgery is the largest unfought TAM and ISRG has a structural advantage, but (c) sustained 50×+ multiples require continued category leadership that competitive entry directly threatens. Not the base case but a real possibility — especially if the May 2026 recall stays contained to one product line.

WHAT HAPPENS

The bull case is that da Vinci 5 unlocks the general-surgery TAM — the unfought ~70% of US soft-tissue procedures still done open or laparoscopically — AND ISRG defends share against Hugo. dV5's force feedback, on-board insufflation, and Iris imaging hit critical mass in 2027-28; placements accelerate to 2,500+/yr. Ion sustains 30%+ procedure growth toward a \$1.5B revenue line. Cardiac clearance translates to real volume. Japan, Korea, Brazil penetrate at pace.

Revenue accelerates above guidance: 17-18% CAGR through FY28, moderating to 12% by FY30. Operating margin expands toward 34% as the high-margin instrument/service razor outpaces systems. FCF reaches ~\$6.9B by FY30. The market doesn't compress the multiple because growth re-accelerates — ~52× FCF sustains. Exit-multiple SOTP: \$269B op EV + \$8.63B cash = \$278B equity ÷ 348M shares = ~\$798/share. +131% from spot.

Probability 5%

Outside-OR + China + AI; 62× re-rate.

WHY 5%

Joint conditional: outside-OR / ASC deployment scales (40% conditional on a dV5 compact form factor) AND China crosses \$2B by FY30 (trimmed to ~20-25% conditional — MicroPort's Toumai, the leading domestic system with 200+ installations and mid-2026 EU access, contests the uncontested-penetration assumption) AND AI / autonomous-surgery monetization gets real (25% conditional on an FDA approval pathway for semi-autonomous assist) AND the multiple re-rates to 60-65× FCF (40%). Joint now lands at 5% (was 6%). Tail of tails — but ISRG has the installed base, the data moat, and the cash to actually attempt all three simultaneously.

WHAT HAPPENS

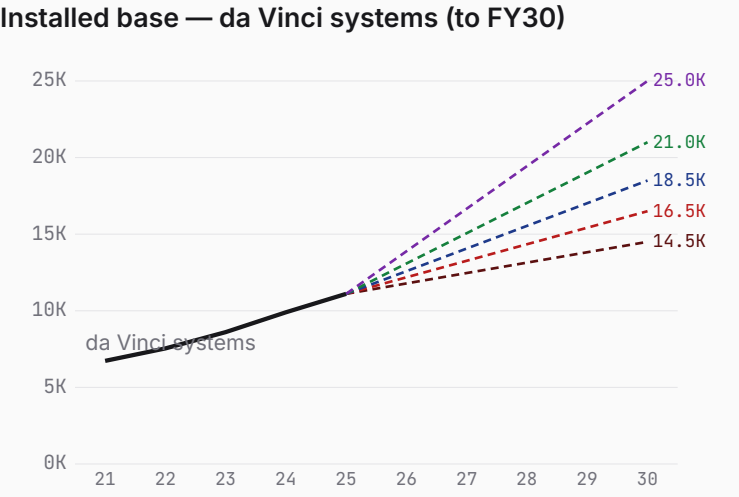
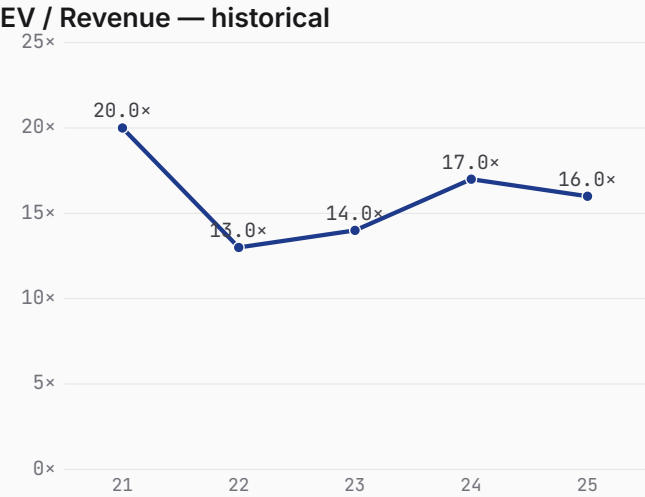
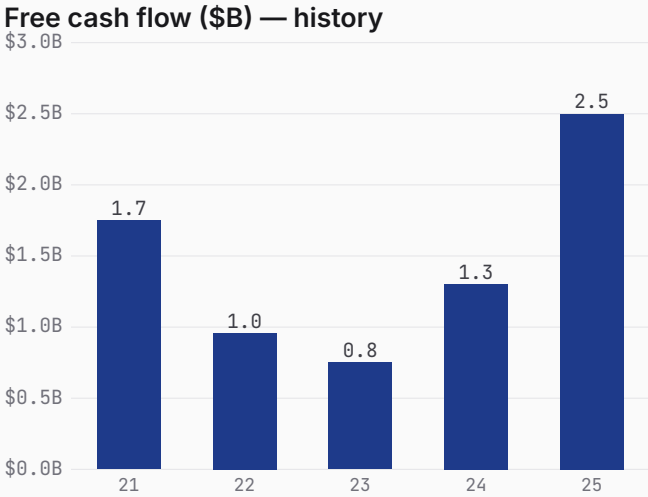
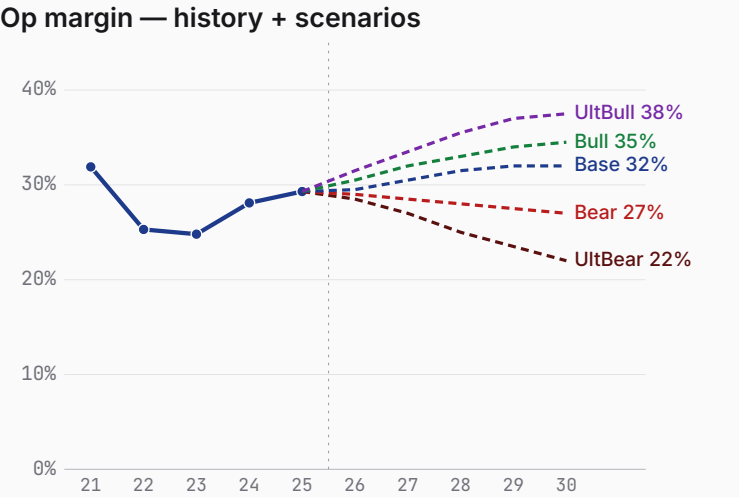
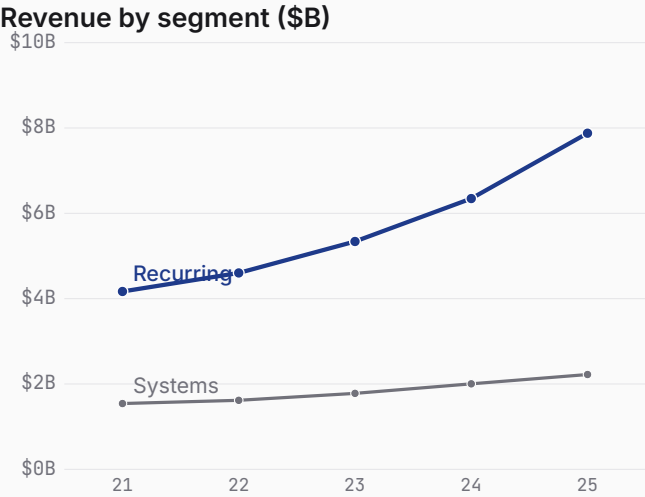
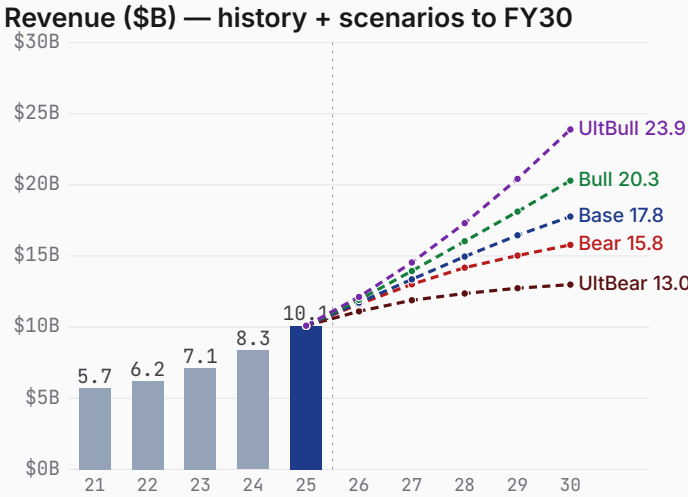
The ultra-bull is ISRG inflecting on three independent axes at once. (1) Outside-OR / clinic deployment: dV5 compact configurations enable ambulatory-surgical-center placements at \$500K-1M (vs \$2.5M traditional), tripling the addressable TAM beyond hospital ORs. (2) China + India scale: China revenue grows from <\$500M past \$2B by FY30 — a leg that must now outrun domestic rivals (MicroPort's Toumai, 200+ commercial installations and EU market access mid-2026); India localized manufacturing enables local-market price points. (3) AI / autonomous surgery: ISRG's data moat (17M+ recorded procedures) becomes training data for semi-autonomous assistance — a new SaaS-like recurring line.

Revenue compounds at ~19% CAGR — \$10.1B → \$25B+ by FY30. Operating margin reaches 36-37% as 90%+-gross-margin software/AI blends in. FCF: \$2.5B → ~\$9B. The market re-rates to growth-software multiples: ~62× FCF on a \$9B base. Exit-multiple SOTP: \$425B op EV + \$8.63B cash = \$434B equity ÷ 345M shares = ~\$1,257/share. +264% from spot. This is the scenario where ISRG stops being "great mature medtech" and becomes "the SaaS-like platform for procedures."

Intuitive Surgical business snapshot

Bear \$274.86 Base \$492.27 Bull \$798.36

FY21–FY25 history + FY26–FY30 scenario projections · fiscal years end Dec 31 · 10-K FY25, Q2 2026 10-Q, FDA filings, medtech competitive analysis



Sources: ISRG 10-K FY25, Q2 2026 8-K/10-Q, Medtronic Hugo FDA clearance Dec 2025 + GS/GYN 510(k) filings Jun 2026, J&J Ottava FORTE/De Novo milestones, surgical-robotics TAM analysis.

Intuitive Surgical 7 Powers · the moat, scored

Bear \$274.86 Base \$492.27 Bull \$798.36

Arena. Soft-tissue robotic surgery — da Vinci's installed base vs. emerging challengers (Medtronic Hugo, J&J Ottawa, CMR Surgical Versius, MicroPort Toumai) for the global multi-specialty minimally-invasive surgery platform standard.

POWER AUDIT
dominant-power durability: high

POWER AUDIT · 7 POWERS

Scale Economies 	11,710 systems installed; instrument/service volume amortizes R&D no challenger matches.
Network Economies 	Surgeon training + case-data flywheel (Case Insights) compounds with the installed base.
Counter-Positioning 	Challengers catch up on model, not displace it; weak collateral-damage barrier.
Switching Costs · thesis leans here 	Sunk capital + surgeon retraining + hospital workflow lock-in; razor/blade recurring revenue.
Branding 	da Vinci is the category synonym; surgeon and hospital default.
Cornered Resource 	Deep patent estate (many now expiring) + the installed base itself as a distribution asset.
Process Power 	Two decades of manufacturing/service learning-curve and reliability data.

PEER SET

Medtronic (Hugo) FDA-cleared (urology) Dec 2025; GS + GYN 510(k)s filed Jun 2026; bundled into a broad device channel.	10% gr · 25% mgn · ~17x EV/EBIT
J&J MedTech (Ottawa) FORTE pivotal met May 2026; FDA De Novo grant reported Jul 2026; Ethicon stapling tie-in.	6% gr · 28% mgn · ~16x EV/EBIT
CMR Surgical (Versius) Modular, lower-cost; OUS traction.	30% gr · private
MicroPort MedBot (Toumai) Leading Chinese domestic system — 200+ global commercial installations, 300+ orders; EU market access mid-2026.	

THREAT VECTORS · FALSIFIERS

Counter-Positioning · Medtronic / J&J falsifier: ISRG procedure growth < 10% for 2 consecutive years while a rival's installed base passes 1,000 systems.
Counter-Positioning · MicroPort (Toumai) + domestic Chinese entrants falsifier: ISRG China revenue declines YoY for 2 consecutive years, or Toumai's installed base outside China passes 1,000 systems.
Switching Costs · Hospitals (capital pressure) falsifier: da Vinci placement mix shifts to lease/usage > 50%, signaling pricing-power erosion.

COMPETITIVE READ

A wide, durable moat anchored on switching costs and scale — the basis for the DCF's premium terminal multiple; challengers are real but years from the installed-base and brand barriers. Watch procedure growth as the erosion gauge.

Intuitive Surgical show your work

Bear \$274.86 Base \$492.27 Bull \$798.36 · Weighted \$504.14 (+33.1%)

ULTRA BEAR					BEAR					BASE					BULL					ULTRA BULL				
\$142.15					\$274.86					\$492.27					\$798.36					\$1257.39				
ASSUMPTIONS					ASSUMPTIONS					ASSUMPTIONS					ASSUMPTIONS					ASSUMPTIONS				
5y revenue CAGR (%)					+5.2					+9.3					+12.0					+15.0				
Year-5 op margin (%)					22.0					27.0					32.0					34.5				
WACC (%)					10.5					9.0					8.0					7.5				
Exit FCF multiple (x)					28					33					45					52				
Installed base FY30					14,500					16,500					18,500					21,000				
Starting revenue (\$B)					10.10					10.10					10.10					10.10				
Scenario probability (%)					6					25					48					16				
5-YEAR DCF · \$B					5-YEAR DCF · \$B					5-YEAR DCF · \$B					5-YEAR DCF · \$B					5-YEAR DCF · \$B				
FY	Rev	Margin	FCF	PV FCF	FY	Rev	Margin	FCF	PV FCF	FY	Rev	Margin	FCF	PV FCF	FY	Rev	Margin	FCF	PV FCF	FY	Rev	Margin	FCF	PV FCF
FY26	11.11	+28%	+2.50	+2.26	FY26	11.61	+29%	+2.85	+2.62	FY26	11.72	+30%	+3.20	+2.96	FY26	11.92	+31%	+3.65	+3.40	FY26	12.12	+32%	+3.82	+3.57
FY27	11.89	+27%	+2.45	+2.01	FY27	13.01	+28%	+3.13	+2.63	FY27	13.36	+31%	+3.70	+3.17	FY27	13.94	+32%	+4.55	+3.94	FY27	14.54	+34%	+4.88	+4.26
FY28	12.36	+25%	+2.30	+1.71	FY28	14.18	+28%	+3.32	+2.56	FY28	14.96	+32%	+4.20	+3.33	FY28	16.04	+33%	+5.45	+4.39	FY28	17.31	+36%	+6.20	+5.06
FY29	12.73	+24%	+2.13	+1.43	FY29	15.03	+28%	+3.40	+2.41	FY29	16.45	+32%	+4.55	+3.34	FY29	18.12	+34%	+6.15	+4.61	FY29	20.42	+37%	+7.55	+5.76
FY30	12.99	+22%	+1.98	+1.20	FY30	15.78	+27%	+3.50	+2.27	FY30	17.77	+32%	+4.85	+3.30	FY30	20.29	+35%	+6.85	+4.77	FY30	23.89	+38%	+9.05	+6.45
Σ PV explicit FCF					+8.61					+12.50					+21.09					+25.11				
+ PV terminal (g 2.0%)					33.65					75.07					248.11					400.06				
EQUITY BUILD · \$B & PER SHARE					EQUITY BUILD · \$B & PER SHARE					EQUITY BUILD · \$B & PER SHARE					EQUITY BUILD · \$B & PER SHARE					EQUITY BUILD · \$B & PER SHARE				
Operating EV					\$42.26B					\$87.57B					\$269.20B					\$425.17B				
+ Cash & investments					\$8.63B					\$8.63B					\$8.63B					\$8.63B				
= Equity value					\$50.89B					\$96.20B					\$277.83B					\$433.80B				
FY30 shares (M)					358					350					348					345				
= Expected per share					\$142.15					\$274.86					\$798.36					\$1257.39				
FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT					FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT					FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT					FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT					FUTURE FAIR VALUE · IF SCENARIO PLAYS OUT				
+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y		+5y	+10y	+15y	+20y	
\$234.18	\$385.81	\$635.60	\$1,047.11		\$422.91	\$650.69	\$1,001.17	\$1,540.43		\$723.31	\$1,062.77	\$1,561.56	\$2,294.45		\$1,146.15	\$1,645.45	\$2,362.25	\$3,391.31		\$1,763.55	\$2,473.48	\$3,469.18	\$4,865.70	
0.62×	1.02×	1.68×	2.76×		1.12×	1.72×	2.64×	4.07×		1.91×	2.81×	4.12×	6.06×		3.03×	4.34×	6.24×	8.95×		4.66×	6.53×	9.16×	12.84×	

Intuitive Surgical

People · Opportunity · Context · Deal

Bear \$274.86 Base \$492.27 Bull \$798.36

Underwrite the position the way a venture investor underwrites a round — across all four legs, public or private. **People** is the leg scored here (observable only); Opportunity, Context and Deal cross-reference the rest of the memo.

PEOPLE · WHO RUNS / OWNS / FUNDS IT

Dave Rosa (CEO since Jul 2025)

1%insider ownership

LOWkey-person risk

Capital allocation (realized)
Net cash, zero debt, no dividend — a self-funded compounder: reinvests first (R&D +~15% YoY 2024 + manufacturing capex), buys back opportunistically (~\$7.95B cumulative, \$5B authorization refreshed), and grows organically (build-not-buy, no transformative M&A).

Incentive alignment
Pay is ~85-89% equity, performance-weighted: PSUs pay 0-125% of target over 3 years on relative TSR + da Vinci/Ion procedure-volume growth + relative operating margin. Guthart FY2024 ~\$18.2M (mostly stock, \$0 options).

GOVERNANCE FLAGS

- single-class (one share, one vote); no controlling individual (insiders <1%, ~85% institutional)
- declassified board — directors elected annually; fully-independent committees; say-on-pay ~93% (2025)
- orderly succession: Rosa is a 29-yr internal promotion; Guthart stayed on as Executive Chair
- caveat: with Guthart as Executive Chair the chair is no longer independent (lead independent director)

PEOPLE READ

The cleanest governance of the cohort: single-class, declassified annual elections, ~93% say-on-pay, performance-weighted equity, net-cash discipline, and an orderly internal CEO handoff (Rosa) — institutionalized, not person-dependent. Mild demerits: low insider skin-in-the-game and a now non-independent (Executive) chair.

THE FOUR LEGS

People

observable composite · 1–5

Opportunity

The scenario distribution · the business snapshot · the Arthur Indicator (ISRG 13.8 — orange after the derating, no longer deep-red)

Context

The 7 Powers analysis (Power Audit)

Deal

Open-market common — entry price weighed against the probability-weighted fair value (the +46% finding), then position-sized.

Intuitive Surgical

supporting analysis · disclaimers · glossary

Bear \$274.86 Base \$492.27 Bull \$798.36

PUSHBACK · WHY THE BASE CASE IS TOO HARSH

- 1

Moat is real and measurable.
11,710 installed base + 17M procedures + 26-year FDA record. Hugo has one US specialty cleared (urology; GS/GYN filed). The asymmetry is enormous.
- 2

Razor-blade is the business.
78% of FY25 revenue recurring (instruments + service). Compounds on installed-base growth even if system pricing compresses.
- 3

Balance sheet is a fortress.
\$8.63B cash, zero debt. Buybacks at scale: \$1.1B retired in Q1 2026, ~\$0.4B more in Q2. Capital return finally at scale.
- 4

da Vinci 5 is reaccelerating.
dV5 placements 246 in Q2 2026 after 232 in Q1 (147 a year ago). Force feedback is a step-function, not incremental.
- 5

Procedure growth is structural.
16% Q2 procedure growth on already-trained surgeons. Grows with aging + minimally-invasive penetration. Hard to bend the curve.

FALSIFICATION TRIGGERS

Bear validation

Hugo GS/GYN cleared before mid-2027 or > 15% of US new placements · ISRG China revenue declines YoY · dV5 placements < 200/qtr · procedure growth < 10% · another Class I recall on a core line · forward P/FCF < 30×

Bull validation

dV5 placements > 300/qtr · Ion procedure growth > 40% · cardiac volume > 5,000/yr · general-surgery share grows materially · forward P/FCF > 50× (Q2: dV5 246, Ion +36%)

Reframe needed

If Stryker / J&J announce a credible cross-platform alliance against ISRG, OR if ISRG acquires a major outside-OR or AI surgical asset (>\$5B deal), the moat / TAM framing changes materially

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Forward-looking statements.

Scenarios, valuations, projections, and any forward-looking statements involve substantial assumptions and uncertainty. Actual results may differ materially from those projected. Past performance is not indicative of future results. The model is a model; reality is not.

Author may hold positions.

The author may own, intend to acquire, or hold short exposure to \$ISRG or related securities at any time. Positions may change without notice and without an update to this document. Do not assume the author's positions match the tone of the analysis.

Do your own research.

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GLOSSARY · CONCEPTS REFERENCED IN THE NARRATIVE

da Vinci 5 — ISRG's 5th-gen surgical robot (2024). Force feedback, on-board insufflation, Iris imaging. Placements accelerating — 246 in Q2 2026 after 232 in Q1.

Ion — ISRG's endoluminal robot for minimally invasive lung biopsies. Q2 2026 procedure growth +36% YoY. Separate installed base.

Hugo (Medtronic) — Medtronic's competing soft-tissue robot. FDA-cleared Dec 2025 for urology — first credible US soft-tissue competitor in 25 years. GS + GYN 510(k)s filed Jun 2026.

Ottava (J&J) — J&J's competing soft-tissue robot. FORTE pivotal endpoints met May 2026; FDA De Novo grant reported Jul 2026. Adjacency to J&J's Monarch.

Installed base — da Vinci systems in service worldwide. 11,710 at Jun 2026 (~+12% YoY). The denominator that drives recurring-revenue compounding.

Recurring revenue mix — Revenue from instruments, accessories, and service (vs one-time systems). FY25 ~78%. The razor-blade lock-in.